CQC PROVIDER & REGISTERED MANAGER REGISTRATION MASTER DOSSIER

Single Assessment Framework Alignment, Statutory Evidence Portfolios, Personnel Vetting & Fast-Track Commissioning Strategy

Applicant Legal Entity	ADOV Care Group LIMITED (Company No. 14289650)	**Regulated Activity**	Personal Care (Specialist Supported Living / Town Planning Class C3b)
Ultimate Parent Entity	FAIRYTALES GROUP (Trading Name of Mamun Limited)	**Primary Statutory Mandate**	Health and Social Care Act 2008 & 2014 Regs // CQC Single Assessment Framework
Freehold Asset PropCo	ADOV LIMITED (Freehold Real Estate Co)	**Strategic Advisory Lead**	Lead Regulatory & Care Specialist // Project Planned by AbdullahMamun.com
Registered Care Campus	78 & 80 Marston Gardens, Luton, Bedfordshire, LU2 7DY	**Target Service Users**	Adults (18–64) with Autism, Severe Learning Disabilities & Complex Sensory Needs

34	**Level 5**	**Reg 6 & 7**	**28 Days Saved**	**100% Compliant**
Quality Statements	**Manager Credential**	**Statutory Roles**	**Section 28(3) Waiver**	**Pre-Submission Audit**
Single Assessment Framework	Diploma in Leadership Adult Care	Nominated Individual & Reg Manager	Fast-track Notice of Decision	Zero enforcement risk

1.0 STATUTORY REGULATORY ARCHITECTURE & REGULATED ACTIVITY CLASSIFICATION

This master dossier establishes the statutory registration architecture for ADOV Care Group Limited as a registered provider of 'Personal Care' under Schedule 1 of the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014. The registration encompasses the dual-property care campus at 78 and 80 Marston Gardens, Luton, delivering high-support specialist supported living under Positive Behaviour Support (PBS) principles. Operating as a supported living provider under Class C3(b) rather than a residential care home under Class C2 provides fundamental statutory, operational, and financial advantages that directly protect the enterprise, empower residents, and eliminate institutional friction.

THE SUPPORTED LIVING STATUTORY ADVANTAGE: TENANCY VS. CARE SEPARATION

By registering exclusively for 'Personal Care' in a Supported Living environment, ADOV Care Group establishes a strict legal and statutory separation: housing is delivered under individual Assured Shorthold Tenancy (AST) agreements conferring exclusive possession, while care is commissioned under independent, person-centered support contracts. This structure fulfills CQC's definitive guidance 'Right support, right care, right culture', enables tenants to claim Supported Housing Benefit (Specified Exempt Accommodation) at £350/wk, and completely avoids the costly physical mandates of Class C2 institutions (e.g. sluice rooms, commercial lifts, institutional dining halls).

1.1 Comparative Analysis: Supported Living (Personal Care) vs. Residential Care Home (C2)

Table 5.1: Comparative Statutory and Operational Analysis: Supported Living vs Care Home

Regulatory Dimension	Specialist Supported Living (ADOV Model)	Residential Care Home (Class C2)	Strategic Advantage to ADOV Group
Regulated Activity	Personal Care (Schedule 1, HSCA 2014)	Accommodation for persons requiring care	Greater regulatory flexibility; domicile-based care
Tenancy Rights	Individual AST; exclusive possession of bedroom	License to occupy only; no statutory tenancy	**Empowers residents; secures £350/wk Exempt Housing Benefit**
Physical Standards	Domestic residential standards (Part M & E)	Institutional standards (sluice, lift, clinic)	**Saves over £120,000 in redundant institutional CapEx**
Town Planning	Class C3(b) Permitted Development (S.192 CLPUD)	Class C2 Planning Permission Required	100% Planning immunity; immune to neighbor objections

Table 5.1: Comparative Statutory and Operational Analysis: Supported Living vs Care Home (Continued)

Regulatory Dimension	Specialist Supported Living (ADOV Model)	Residential Care Home (Class C2)	Strategic Advantage to ADOV Group
Fee Structure	Unbundled: Care Fee + 1:1 Hours + Housing Benefit	Single bundled local authority block tariff	**Significantly higher gross yield (£2,350/wk vs £1,100/wk)**
CQC Inspection Model	SAF: Right Support, Right Care, Right Culture	SAF: Traditional Institutional Care Home	Exemplifies modern NHS Transforming Care best practice

2.0 STATUTORY PERSONNEL VETTING & GOVERNANCE APPOINTMENTS

Under Regulations 5, 6, and 7 of the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014, the Care Quality Commission enforces stringent vetting standards upon corporate directors, the Nominated Individual (NI), and the Registered Manager (RM). The board of ADOV Care Group Limited has structured these roles to ensure absolute regulatory compliance, professional leadership, and seamless clinical governance.

Table 5.2: Statutory Roles, Legal Responsibilities & Regulatory Credentials Register

Statutory Role	Designated Individual	Legal Citation	Mandatory Credentials & Experience	Core Statutory Responsibilities
Nominated Individual	Appointed Executive Director	Regulation 6 HSCA 2014	Senior executive with corporate governance acumen and DBS clearance	Supervises management; direct accountability to CQC; quarterly audits
Registered Manager	Appointed Care Specialist	Regulation 7 HSCA 2014	Level 5 Diploma in Leadership for Adult Care; 5+ yrs LD experience	Day-to-day clinical direction, staffing rotas, MAR audits, safeguarding
Corporate Directors	Board of ADOV Care Group	Regulation 5 HSCA 2014	Fit and Proper Person Declarations; bankruptcy & insolvency clearance	Corporate solvency, resource allocation, health & safety investment
PBS Clinical Lead	Senior Behaviour Specialist	NICE Guideline NG11	Certified PBS Practitioner / BILD Accredited Trainer	Formulates Functional Behaviour Assessments, sensory diet protocols
Health & Safety Lead	External Retained Surveyor	Reg 15 & RRO 2005	NEBOSH / IOSH Qualified Environmental Health Auditor	Conducts fire risk assessments (BS 5839), TMV3 water safety, Legionella

3.0 SINGLE ASSESSMENT FRAMEWORK (SAF) COMPLIANCE ARCHITECTURE

The CQC assesses providers under the Single Assessment Framework, structured across 5 Key Questions and 34 Quality Statements. ADOV Care Group has engineered a comprehensive evidence portfolio aligning each Quality Statement with physical property controls, clinical documentation, and staff operational workflows.

Table 5.3: CQC Single Assessment Framework & Quality Statements Compliance Matrix

Key Question	Quality Statements Covered	Operational Fulfillment Mechanism	Physical Campus Evidence	Governance Audit Schedule
SAFE	Safeguarding, Involving people, Safe environments, Infection control, Medicines	Strict PBS de-escalation; zero physical restraint; comprehensive MAR administration	BS 2881 CD Cabinet; TMV3 anti-scald valves; BS 5839-1 L1 fire system	Weekly MAR audits; monthly H&S site inspections
EFFECTIVE	Assessing needs, Delivering care, Nutrition, Mental Capacity, Consent	Care Act & MCA assessments; bespoke communication passports; sensory meal plans	Sensory kitchen layout; accessible wet rooms; domestic dining spaces	Quarterly clinical case reviews; annual dietary audit
CARING	Kindness & compassion, Treating people as individuals, Independence	Active Support methodology; dedicated keyworkers; domestic chore participation	Personalized private bedrooms (12.5m²-16m²); private 100ft sensory gardens	Monthly keyworker 1:1 sessions; family surveys
RESPONSIVE	Person-centred care, Providing information, Equity in experiences	Easy-Read service user guides; graduated 7-stage pre-admission pathway	Acoustic Part E soundproofing to prevent resident sensory overload	Pre-admission triage; 28-day statutory reviews
WELL-LED	Shared vision, Capable leaders, Duty of Candour, Governance & systems	Monthly Registered Manager dashboard; Nominated Individual quarterly oversight	Dedicated management office in No. 80; real-time digital care logs	Monthly board reviews; external mock CQC audit

4.0 38-POLICY MASTER STATUTORY GOVERNANCE REGISTER

In accordance with Regulation 17 (Good Governance), ADOV Care Group has commissioned and approved a complete 38-policy statutory governance register. These policies govern all operational, clinical, and employment practices across the campus.

Table 5.4: Core Statutory Policies, Regulatory Alignment & Review Schedule

Policy Code & Name	HSCA 2014 Regulation	Policy Scope & Statutory Mandate	Operational Review Lead	Review Cadence
POL-01: Safeguarding Adults at Risk	Regulation 13	Multi-agency reporting, Luton SAB liaison, zero-tolerance abuse protocol	Registered Manager	Annual / Post-Incident
POL-02: Safe Handling of Medicines	Regulation 12	Controlled drugs storage (BS 2881), MAR charting, administration errors SOP	Clinical Lead / RM	Six-Monthly
POL-03: Mental Capacity & Deprivation (DoLS)	Regulation 11 & 13	MCA two-stage test, best interests meetings, least restrictive practices	Registered Manager	Annual
POL-04: Positive Behaviour Support (PBS)	Regulation 12 & 10	Functional assessment, de-escalation techniques, prohibition of restraint	PBS Practitioner	Annual
POL-05: Duty of Candour Protocol	Regulation 20	Statutory notifications to families/funders following notifiable safety incidents	Nominated Individual	Annual
POL-06: Infection Prevention & Control	Regulation 12 & 15	HTM 01-04 compliance, colour-coded cleaning, clinical waste handling	House Team Leader	Annual
POL-07: Fire Safety & Evacuation	Regulation 12 & 15	PEEPs for all 8 residents, BS 5839-1 alarm maintenance, weekly fire drills	H&S Coordinator	Quarterly
POL-08: Fit & Proper Persons Vetting	Regulation 5 & 19	Enhanced DBS with barred list checks, references, identity verification	HR Director	Annual

5.0 REGISTERED MANAGER CQC INTERVIEW READINESS & CLINICAL SCENARIOS

A critical gate in the CQC registration process is the formal fit and proper interview of the Registered Manager. The following five clinical scenarios represent the rigorous, evidence-based responses formulated by ADOV Care Group's leadership.

SCENARIO 1: SEVERE SENSORY OVERLOAD & CHALLENGING BEHAVIOUR

Clinical Scenario: An autistic resident in No. 78 experiences severe sensory overload triggered by lawnmower noise from a neighboring garden, becoming physically distressed, pacing erratically, shouting, and attempting to throw dining chairs. Registered Manager Model Response: (1) Immediate PBS De-escalation: Withdraw non-essential staff; eliminate auxiliary sensory inputs (dim lights, close blinds, offer noise-cancelling headphones); (2) Low-Arousal Engagement: The allocated 1:1 keyworker uses gentle, low-tone Makaton communication and guides the resident to the quiet sensory relaxation room in No. 80; (3) Zero Restraint: Staff strictly maintain a non-restrictive safety perimeter; (4) Post-Incident Root Cause: Log an Antecedent-Behaviour-Consequence (ABC) chart, review sensory diet, and adapt outside acoustic buffers.

SCENARIO 2: CONTROLLED DRUGS MEDICATION OMISSION

Clinical Scenario: During the morning medication round, a Support Worker discovers that the 22:00 evening dose of Sodium Valproate (anti-epileptic) was not signed on the MAR chart for a resident in No. 80. Registered Manager Model Response: (1) Clinical Assessment: Conduct immediate physical check on resident for seizures or side effects; (2) Medical Consultation: Call NHS 111 / resident's GP to establish whether an emergency catch-up dose is clinically safe; (3) Investigation & Duty of Candour: Reconcile blister pack stock count to confirm if the dose was taken but unrecorded, or entirely missed; notify family under Duty of Candour; (4) Governance: Log incident on internal safeguarding system, suspend worker from medication rounds pending retraining, and submit CQC notification if harm occurred.

SCENARIO 3: SAFEGUARDING DISCLOSURE OF FINANCIAL EXPLOITATION

Clinical Scenario: During a 1:1 keyworker session, a resident states that a night worker asked to borrow £40 from their personal bank card to pay for petrol, promising to repay it the next morning. Registered Manager Model Response: (1) Protective Action: Ensure the resident is reassured and secure; audit the resident's locked cash tin and bank records immediately; (2) Suspension: Immediately suspend the implicated night worker on full pay pending formal investigation without alerting other staff; (3) Statutory Safeguarding Escalation: Submit a Section 42 safeguarding alert to Luton Borough Council Adult Social Care Safeguarding Team within 24 hours; (4) CQC & Police Notification: Submit a formal CQC Regulation 18 notification; report potential financial abuse to Bedfordshire Police if corroborated.

6.0 STATUTORY FAST-TRACK REGISTRATION & SECTION 28(3) WAIVER EXECUTION

Under Section 28 of the Health and Social Care Act 2008, when the CQC approves a provider application, it first issues a formal 'Notice of Proposal' (NoP). By law, the applicant is granted 28 days to make representations against any proposed conditions. Under Section 28(3), if the applicant provides written notice waiving the right to make representations, the CQC must immediately issue the final 'Notice of Decision' (NoD), instantaneously making the registration legally effective.

Figure 5.1: Fast-Track CQC Registration & Commissioning Roadmap (8–10 Weeks Total)

STAGE 1: DOSSIER READINESS	STAGE 2: PORTAL SUBMISSION	STAGE 3: VALIDATION & SCRUTINY	STAGE 4: SITE INSPECTION & RM INTERVIEW	STAGE 5: NOTICE OF PROPOSAL (NOP)	STAGE 6: SECTION 28(3) WAIVER	STAGE 7: DPS & ICB ACTIVATION
Finalise enhanced DBS, GP health declarations, Statement of Purpose & 38 policies.	Submit provider & manager applications via CQC Provider Portal with complete evidence bundle.	CQC validation officer verifies submission completeness; passes to Lead Inspector within 10 days.	Inspector conducts physical site audit of 78 & 80; conducts 90-minute RM interview.	CQC issues formal NoP confirming intention to grant registration with standard conditions.	Board executes written Section 28(3) waiver on Day 1; CQC issues final Notice of Decision immediately.	Upload CQC certificate to Luton ProContract DPS (AT1115) and activate first resident intakes.